ANNEX 1

Horizon Europe (HORIZON)

Description of the action (DoA)

Part A

Part B

DESCRIPTION OF THE ACTION (PART A)

COVER PAGE

Part A of the Description of the Action (DoA) must be completed directly on the Portal Grant Preparation screens.

PROJECT *Grant Preparation (General Information screen) — Enter the info.*	
Project number:	101155873
Project name:	Social Prescribing to promote and improve access to health and care services for people in vulnerable situations in Europe
Project acronym:	SP-EU
Call:	HORIZON-HLTH-2024-CARE-04-two-stage
Topic:	HORIZON-HLTH-2024-CARE-04-04-two-stage
Type of action:	HORIZON-RIA
Service:	HADEA/A/03
Project starting date:	fixed date: 1 January 2025
Project duration:	60 months

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PROJECT SUMMARY

Project summary

Grant Preparation (General Information screen) — Provide an overall description of your project (including context and overall objectives, planned activities and main achievements, and expected results and impacts (on target groups, change procedures, capacities, innovation etc)). This summary should give readers a clear idea of what your project is about.

Use the project summary from your proposal.

Social Prescribing (SP) is an innovative solution to bridging the gap between primary health care and non-clinical supports and services within the community. SP enables general practitioners who identify health-related social problems to refer patients to a so-called link worker. The link worker provides personalized support and helps these patients to access community resources by addressing any barriers that may exist. SP is an approach to integrate health and social care and strengthen community orientation. SP thereby mitigates the effects of social determinants of health on health outcomes. However, SP has not been tailored to the needs of people in vulnerable situations yet and the effectiveness of SP to improve their access to health and care services remains unknown.
The overall objective of SP-EU is to assess the potential of SP to promote and improve access to health and care services for people in vulnerable situations, focusing on three primary target groups: LGBTIQ persons, refugees and first-generation immigrants and older adults living alone.
SP-EU follows a mixed-methods approach: SP adaptations will be co-created with the target groups, to adapt them to their specific needs and social contexts. A pragmatic randomized controlled trial will assess the effectiveness of the adapted SP to improve access by randomizing 1,776 patients in eight EU countries to receive SP or care as usual. A qualitative analysis in five European countries will explore enabling and limiting factors to the implementation of SP from different stakeholders' perspectives. Additionally, we will communicate and disseminate results and translate them into policy action.
SP-EU will enable health care systems to implement SP as a scalable, safe, cost-effective and people-centred solution to provide equal access to sustainable care. Thus, SP-EU will enable the transformation of European health care systems to equitable, community-based, people-centred and integrated health care.

LIST OF PARTICIPANTS

PARTICIPANTS

Grant Preparation (Beneficiaries screen) — Enter the info.

Number	Role	Short name	Legal name	Country	PIC
1	COO	Charite	CHARITE - UNIVERSITAETSMEDIZIN BERLIN	DE	999992692
2	BEN	GÖG	GESUNDHEIT OSTERREICH GMBH	AT	928836208
3	BEN	concentris	CONCENTRIS RESEARCH MANAGEMENT GMBH	DE	995658053
4	BEN	UoP	UNIVERSITY OF PLYMOUTH	UK	999854661
5	BEN	WMU	UNIWERSYTET MEDYCZNY IM PIASTOW SLASKICH WE WROCLAWIU	PL	998757300
6	BEN	IDIAPJGol	FUNDACIO INSTITUT UNIVERSITARI PERA LA RECERCA A L'ATENCIO PRIMARIA DE SALUT JORDI GOL I GURINA	ES	994512095
6.1	AE	ICS	INSTITUT CATALA DE LA SALUT	ES	999574913
7	BEN	UNL	UNIVERSIDADE NOVA DE LISBOA	PT	960782479
8	BEN	AAU	AALBORG UNIVERSITET	DK	999904034

PARTICIPANTS

Grant Preparation (Beneficiaries screen) — Enter the info.

Number	Role	Short name	Legal name	Country	PIC
9	BEN	BUW	BERGISCHE UNIVERSITAET WUPPERTAL	DE	999858735
10	BEN	UOXF	THE CHANCELLOR, MASTERS AND SCHOLARS OF THE UNIVERSITY OF OXFORD	UK	999984350
11	BEN	UKE	UNIVERSITAETSKLINIKUM HAMBURG-EPPENDORF	DE	999857474
12	BEN	ACT	ACTIVE GROUP GMBH	DE	933586686
13	BEN	DZZZ	DOM ZDRAVLJA ZAGREB ZAPAD	HR	936773136
14	BEN	UKF	UNIVERSITAETSKLINIKUM FREIBURG	DE	999881918
15	BEN	EuroHealthNet	EUROHEALTHNET ASBL	BE	998095857
16	BEN	UGent	UNIVERSITEIT GENT	BE	999986096
17	BEN	NASP	THE NATIONAL ACADEMY FOR SOCIAL PRESCRIBING	UK	880775230
18	BEN	WONCA Europe	STICHTING WONCA EUROPE	SI	996295440
19	BEN	DSIH	DANSK SELSKAB FOR INDVANDRERSUNDHED	DK	877759985
20	BEN	SB	Schwulenberatung Berlin gemeinnutzige GmbH	DE	930456787
21	AP	UBERN	UNIVERSITAET BERN	CH	999976493
22	AP	SRK-Bern	Schweizerisches Rotes Kreuz Kanton Bern, Region Seeland - Berner Jura	CH	877851456

LIST OF WORK PACKAGES

Work packages

Grant Preparation (Work Packages screen) — Enter the info.

Work Package No	Work Package name	Lead Beneficiary	Effort (Person-Months)	Start Month	End Month	Deliverables
WP1	Coordination, Project and Data Management (1/4)	1 - Charite	20.70	1	12	D1.1 – Rules of Procedure D1.2 – Data Management Plan - V1
WP2	Coordination, Project and Data Management (2/4)	1 - Charite	31.70	13	30	D2.1 – Ethics Monitoring Report D2.2 – Early-Stage Career Support Report D2.3 – Data Management Plan - V2
WP3	Coordination, Project and Data Management (3/4)	1 - Charite	31.70	31	48	D3.1 – Ethics Monitoring Update D3.2 – Gender and Diversity Report D3.3 – Data Management Plan - V3
WP4	Coordination, Project and Data Management (4/4)	1 - Charite	20.70	49	60	D4.1 – Open Science Report D4.2 – Governance Report D4.3 – Data Management Plan - V4
WP5	Co-Creation of Social Prescribing Intervention	8 - AAU	27.10	1	12	D5.1 – Co-Creation Report
WP6	Digital Linkworking Tool Development	12 - ACT	16.80	1	12	D6.1 – ALTAI-Report D6.2 – Prototype of Digital Tool
WP7	Digital Linkworking Tool Support	12 - ACT	3.00	13	36	D7.1 – Digital Tool Documentation D7.2 – Digital Tool
WP8	RCT Preparation incl. Ethics	1 - Charite	53.96	1	20	D8.1 – Study Initiation Package D8.2 – GP Practice Recruitment Report
WP9	RCT Implementation (1/2)	1 - Charite	247.86	16	30	D9.1 – Midterm Recruitment Report
WP10	RCT Implementation (2/2)	1 - Charite	124.50	31	43	D10.1 – Final Follow-Up Report
WP11	RCT Analysis	1 - Charite	32.00	31	60	D11.1 – Report on the status of posting trial results

Work packages

Grant Preparation (Work Packages screen) — Enter the info.

Work Package No	Work Package name	Lead Beneficiary	Effort (Person-Months)	Start Month	End Month	Deliverables
						D11.2 – Anonymized Trial Data
WP12	RCT Process Evaluation	4 - UoP	56.40	19	48	D12.1 – Trial Fidelity Report
WP13	Health Economics Assessment	9 - BUW	12.00	5	60	D13.1 – Theory of Change Model D13.2 – Health Economic Publication Update
WP14	Comparative Qualitative Study	10 - UOXF	84.70	13	36	D14.1 – Qualitative Recruitment Report D14.2 – Qualitative Study Publication Update
WP15	Co-Interpretation of Results	7 - UNL	24.00	49	60	D15.1 – Co-Interpretation Report D15.2 – Co-Creation Publication Update
WP16	Dissemination, Exploitation, Communication & Policy (1/4)	17 - NASP	9.40	1	12	D16.1 – Website incl. Knowledge Hub D16.2 – Communication and Dissemination Plan - V1
WP17	Dissemination, Exploitation, Communication & Policy (2/4)	17 - NASP	10.60	13	30	D17.1 – Report on the Use of Communication Channels D17.2 – Communication and Dissemination Plan - V2
WP18	Dissemination, Exploitation, Communication & Policy (3/4)	15 - EuroHealthNet	18.40	31	48	D18.1 – Report on the Established Dissemination Networks D18.2 – Explanatory Film on SP-EU for the Public D18.3 – Communication and Dissemination Plan - V3
WP19	Dissemination, Exploitation, Communication & Policy (4/4)	15 - EuroHealthNet	12.50	49	60	D19.1 – Report on Policy Roundtables D19.2 – Final Exploitation Plan

Work packages

Grant Preparation (Work Packages screen) — Enter the info.

Work Package No	Work Package name	Lead Beneficiary	Effort (Person-Months)	Start Month	End Month	Deliverables
						D19.3 – Communication and Dissemination Plan - V4

Work package WP1 – Coordination, Project and Data Management (1/4)

Work Package Number	WP1	Lead Beneficiary	1 - Charite
Work Package Name	Coordination, Project and Data Management (1/4)		
Start Month	1	End Month	12

Objectives
To kickstart the project and set up an effective governance structure - Set up bodies and regulations for an effective project management (T1.1-T1.5) - Set up regulations and plans for data management and publications (T1.6, T1.7) This WP contributes to all major objectives (Obj.1-Obj.5) of SP-EU.

Description
Lead (L): Charité; Participants (P): concentris and all partners T1.1 Develop a code of conduct and rules of procedures for the project governance (M1-3) L: Charité; P: concentris and all partners We will develop a code of conduct and rules of procedures for general assemblies, executive board, project management office and all additional boards and committees. The code of conduct and the rules of procedures will be ratified by the first general assembly in T1.2. These regulations will be made available to all partners. T1.2 Plan and conduct a kick-off general assembly (M1-3) L: Charité; P: concentris and all partners We will hold the first general assembly (GA) of SP-EU as kick-off meeting in Berlin. During the GA, we will agree on the code of conduct and rules of procedures (T1.2) and prepare the first year of SP-EU in detail. The general assembly will elect the executive board (EB). This kick-off meeting will be used as starting point for communication work in WP16. This task is a prerequisite for T1.3 The successful conduction of the first general assembly as kick-off of the project will be our first milestone (MS1). T1.3 Set up continuous support of collaboration in SP-EU and steering of the project (M4-12) L: Charité; P: concentris and all partners Following the 1st GA we will constitute the executive board (EB) which will meet online at least each quarter of a year. We will foster the open exchange between all participants also in regular video conferences and meetings of the consortium. The EB will continuously monitor the progress in all active work packages in close collaboration with the work package leaders using the project-intranet (incl. a web-based project management tool). The executive board will take measures to enhance collaboration and if necessary, mitigation measures if there is a risk of not meeting the work plan goals. T1.4 Set up continuous project, contract, financial and risk monitoring (M1-12) L: Charité; P: concentris, all partners The coordinator will monitor the progress and quality of SP-EU regarding work packages, tasks, deliverables, milestones, and risks continuously together with the Executive Board supported by concentris and in compliance with the obligations under the EC-GA and the Consortium Agreement. We will screen for early potential deviations from the work plan and discuss mitigation measures together with work package and task leads and partners involved. These mitigation measures might include reallocation of lump sum funding from one partner to another as well as reporting and amendments of the grant agreement to the European Commission. T1.5 Set up continuous ethics and diversity monitoring (M1-12) L: Charité; P: all partners Charité will coordinate all ethics related aspects of the different project parts. All ethics related documents will be stored in the project's intranet. All partners will communicate their ethics related tasks to the Charité as coordinator. We will set up an ombudsman, where each partner as well as participants in the different study parts of SP-EU can report shortcomings regarding ethics including any incidents regarding discrimination by gender or diversity background. We will set up a plan how to handle such incidences related to the locally gender and equality plans of all institutions. T1.6 Write the initial data management plan (M4-6) L: Charité; P: all partners This task will be conducted by the coordinator team at Charité in collaboration with all partners. Based on the data management as sketched in this proposal, we will write an initial data management plan for SP-EU as a whole. This data management plan will include all aspects of data sharing between the partners of SP-EU as well as sharing of data with the public. The data management plan will use the according template by the EC.

T1.7 Draft a scientific publication plan (M5-8) L: Charité; P: all partners
This task will be conducted by the coordinator team at Charité in close collaboration with the communication and dissemination teams as well as with all partners. We will draft a plan for all scientific publications, we expect to be written throughout SP-EU. This plan will include a responsible researcher for each publication and who will contribute how. The plan will be designed to allow all partners to thrive scientifically with visible scientific output and especially take into account the needs and wishes of early career stage researchers.

Work package WP2 – Coordination, Project and Data Management (2/4)

Work Package Number	WP2	**Lead Beneficiary**	1 - Charite
Work Package Name	Coordination, Project and Data Management (2/4)		
Start Month	13	**End Month**	30

Objectives
To enable a fruitful & effective cooperation of all partners and support early-stage researchers to thrive - Support early-stage researchers with a support plan and PhD network (T2.1, T.2.2) - Conduct two general assemblies (T2.3) - Support collaborative, efficient and ethically sound research (T2.4, T2.5) This WP contributes to all major objectives (Obj.1-Obj.5) of SP-EU.

Description
L: Charité; P: concentris & all partners/ WP2 continues WP1 (split due to lump sum funding) T2.1 Develop an early career stage researcher support plan (M13-18) L: Charité This task will be conducted by the coordinator team at Charité in close collaboration with the early career stage researchers from all partners. This plan will include mentoring programs, skill development workshops, and opportunities for networking. This support will contribute to the early career stage researchers' professional development and ensure their success within the project. T2.2 Found the European Social Prescribing PhD-Network (M15-30) L: Charité Several partners of SP-EU will contribute with mainly externally funded PhD students to the project. We will connect these students together and assure supervision by PIs from different partners. The mutual exchange between these PhD students will be very important to assure sustainability. With these PhD students in the project as core, we will build up a European Social Prescribing PhD-Network which is open to all PhD students in the field of social prescribing. The PhD students will organize themselves in this network supported by the SP-EU consortium. T2.3 Continuous support of collaboration and steering of the project (M13-30) continues T1.3 T2.4 Continuous project, contract, financial and risk monitoring (M13-30) continues T1.4 T2.5 Continuous ethics and diversity monitoring (M13-30) continues T1.5

Work package WP3 – Coordination, Project and Data Management (3/4)

Work Package Number	WP3	**Lead Beneficiary**	1 - Charite
Work Package Name	Coordination, Project and Data Management (3/4)		
Start Month	31	**End Month**	48

Objectives
To enable a fruitful and effective cooperation of all partners and support all team members to be aware regarding gender and diversity - Enable open science in SP-EU by supporting all members of the consortium (T3.1)

- Conduct two general assemblies (T3.2)
- Support collaborative, efficient and ethically sound research (T3.2, T3.3, T3.4)

This WP contributes to all major objectives (Obj.1-Obj.5) of SP-EU.

Description

L: Charité; P: concentris & all partners / WP3 continues WP2 (split due to lump sum funding)

T3.1 Support open science practices (M31-42) L: Charité
This task will be conducted by the coordinator team at Charité in close collaboration with all partners. We will provide training sessions on open science practices to ensure all team members understand their roles and responsibilities. This will promote transparency, reproducibility, and the ethical conduct of research within the project. We will identify needs of the partners and how they can be met, e.g., by additional individual training.

T3.2 Continuous support of collaboration and steering of the project (M31-48) continues T2.3

T3.3 Continuous project, contract, financial and risk monitoring (M31-48) continues T2.4

T3.4 Continuous ethics and diversity monitoring (M31-48) continues T2.5

Work package WP4 – Coordination, Project and Data Management (4/4)

Work Package Number	WP4	**Lead Beneficiary**	1 - Charite
Work Package Name	Coordination, Project and Data Management (4/4)		
Start Month	49	**End Month**	60

Objectives

To reach all results of SP-EU in time to achieve the best possible impact
- Support collaborative, efficient and ethically sound research (T4.1, T4.2, T4.3)
- Make all results and data available (T4.4)
- Conduct a final general assembly (T4.5)
- Conduct a final symposium to close the project publicly noticeable (T4.5)

This WP contributes to all major objectives (Obj.1-Obj.5) of SP-EU.

Description

L: Charité; P: concentris & all partners / WP4 continues WP3 (split due to lump sum funding)

T4.1 Continuous support of collaboration and steering of the project (M49-60) continues T3.2

T4.2 Continuous project, contract, financial and risk monitoring (M49-60) continues T3.3

T4.3 Continuous ethics and diversity monitoring (M49-60) continues T3.4

T4.4 Coordination of publishing of all project-related data (M49-60) L: Charité; P: all partners
This task will be conducted by the coordinator team at Charité in close collaboration with partners, especially those hosting data from the different parts of SP-EU. We will ensure that all collected data from SP-EU will be published openly available as stated in data management plan. We will link these data on the website of SP-EU and will use these links as well in all publications which result from SP-EU.

T4.5 Organizing a symposium attached to the final general assembly (M55-60) L: Charité; P: concentris and all partners
Attached to the final general assembly, we will conduct a one-day symposium to present all major results of SPEU to the scientific and public audience. This symposium will give time to discuss the results from the perspectives of different stakeholders such as researchers, patients, people in vulnerable situations, health care providers and policy makers.

Work package WP5 – Co-Creation of Social Prescribing Intervention

Work Package Number	WP5	Lead Beneficiary	8 - AAU
Work Package Name	Co-Creation of Social Prescribing Intervention		
Start Month	1	End Month	12

Objectives
To adapt existing social prescribing interventions to the needs of a) LGBTIQ persons, b) refugees and first-generation immigrants and c) older adults living alone (T5.1-T5.5) This WP contributes to Obj.1, KPI1.1, KPI1.2

Description
L: AAU; P: UBERN, UNL, Charité, SB, DSIH, SRK-Bern, WONCA Europe and all RCT hubs This work package will be conducted by all RCT hubs jointly with specific tasks for AAU, Charité, UBERN and UNL as co-creation pioneer hubs and SB, DSIH and SRK-Bern as patient representatives. T5.1 Conduct a review of social prescribing manuals and protocols (M1-3) L: UNL; P: all partners This task will be led by the team from UNL receiving help from all other partners. We will assemble the existing manuals and protocols on social prescribing interventions and condense the information in it. We will focus especially on training of GPs, training of link workers, recruitment of patients and outcome measurements. The results of T5.1 feed then into T5.3 T5.2a,b,c Conduct a workshop with each group a) LGBTIQ persons, b) refugees and first-generation immigrants and c) older adults living alone with the aim to develop three case vignettes of persons belonging to these groups who might benefit from social prescribing (M1-3) L: AAU; P: Charité, UBERN, SB, DSIH, SRK-Bern This task will be led by the teams from Charité (T5.2a), AAU (T5.2b) and UBERN (T5.2c). Each team will prepare, organize and conduct a workshop with representatives of the according groups. In these workshops, we will introduce the concept of SP in general and the concept of SP-EU. In these workshops we will write up case vignettes of exemplary persons belonging to these groups who might benefit from SP. The results of T5.2 feed then into T5.3. T5.3a,b,c Conduct a series of workshops with representatives of a) LGBTIQ persons, b) refugees and firstgeneration immigrants and c) older adults living alone to adapt core components of SP interventions (M4-7) L: AAU; P: Charité, UBERN, SB, DSIH, SRK-Bern This task will be led by the teams from Charité (T5.3a), AAU (T5.3b) and UBERN (T5.3c). We will apply the case vignettes from T5.2 on the core components of SP interventions as assembled in T5.1. In two longer workshops with each group, we will discuss how these core components have to be adapted to fit the needs of persons belonging to the respective groups. This will lead to adaptations of these core components, e.g., adaptations of link worker trainings to include racism-sensibility training. Between the workshops, the teams will consult with each other and compare the progress and intermediary results. T5.3 will result in preliminary adaptations of the core component of social prescribing interventions to the needs of the three vulnerable groups and feed into T5.4. T5.4 Discussion of the preliminary adaptation with local advisory boards in all hubs (M8-9) L: AAU; P: Charité, UBERN and all RCT hubs This task will be led by the team from AAU with support by Charité and UBERN. We will present and discuss the preliminary adaptations of the social prescribing intervention in each of the RCT hubs with the local advisory board. Each local advisory board consists of a representative of each of the three groups and a representative of local GPs. The advisory boards will meet with a researcher from the hub and prepare feedback on the proposed adaptations. This feedback will then feed T5.5. T5.5 Consensus conference on adapted SP interventions for a) LGBTIQ persons, b) refugees and firstgeneration immigrants and c) older adults living alone (M10-12) L: AAU; P: Charité, UBERN, SB, DSIH, SRK-Bern, WONCA Europe This task will be led by the team from AAU with support by Charité and UBERN. In a final consensus conference, we will bring together the feedback from all local advisory boards. The consensus conference will consist of representatives of each of the three groups, of health care providers and of all RCT hubs. We will jointly discuss the feedback and finally decide following the consent principle on the adaptations of SP interventions to be used in SP-EU. This final decision

will be written up and made available as manual and protocol adaptations to be used in the other work packages of SP-EU. This is the second milestone of SP-EU (MS2).

Work package WP6 – Digital Linkworking Tool Development

Work Package Number	WP6	**Lead Beneficiary**	12 - ACT
Work Package Name	Digital Linkworking Tool Development		
Start Month	1	**End Month**	12

Objectives
To develop a web-based open-source technical solution to support link workers to deliver adapted SP. - To develop the tool based on the needs of the stakeholders (T6.1, T6.3) - To assess the trustworthiness of the tool (T6.2) - To test the tool and write a documentation (T6.4, T6.5) This WP contributes to Obj.1, KPI1.2 and Obj.2

Description
L: ACT T6.1 Analysis of requirements with stakeholders (M1-2) L: ACT ACT will analyse the requirements for the linkworking tool receiving input from all partners of SP-EU. This will lead to a product backlog used in T6.2. T6.2 ALTAI-Assessment (M3-6) L: ACT ACT will assess the digital tool by using the Assessment List for Trustworthy Artificial Intelligence. For all identified risks, measures will be taken to minimize the risk. This task will lead to D6.1. T6.3 Develop digital linkworking tool (M2-7) L: ACT Based on the product backlog from T6.1, Active Group will develop an open source linkworking tool. Active Group will set up development infrastructure including source-code management, issue tracking, and a reproducible build environment and continuous-integration process - assemble a sample data set for presentation in the tool, - implement a backend for the tool, and provide it with the sample data set, - implement a frontend for linkworkers that allows finding local offerings. Parallel to the backend/frontend development, Active Group will: - implement a scraper for local offerings from public GIS sources, - implement a curation frontend for the system, - implement an AI-based curator for these sources. This task will result in a demonstrator which is D6.2. The results of T6.3 feed into T6.4 and T6.5. T6.4 Testing the linkworking tool (M6-10) L: ACT The tool developed in T6.3 will be tested iteratively with users out of the consortium. This will help to further refine the digital tool. The results of T6.4 feed into T6.5. T6.5 Documentation of the tool (M7-12) L: ACT Based on the tool developed in T6.3 and tested in T6.4, ACT will write a documentation of the digital tool which can be used by the link workers in the RCT and then afterwards by the public. The results of T6.5 feed into WP7.

Work package WP7 – Digital Linkworking Tool Support

Work Package Number	WP7	**Lead Beneficiary**	12 - ACT
Work Package Name	Digital Linkworking Tool Support		
Start Month	13	**End Month**	36

Objectives
To support the use of the web-based open-source technical solution during the trial - Train and support link workers to use the tool (T7.1, T7.2) - Prepare the open source publication of the tool (T7.3, T7.4) This WP contributes to Obj.2.

Description
L: ACT T7.1 Develop training material on the tool for link workers (M13-16) L: ACT Based on the documentation from T6.5, ACT will develop training material for link workers regarding the digital tool. This training material will feed into the general training material development in T8.7. Thus, this step will be closely synchronized with the partners from UoP and UOXF. The results of T7.1 feed then into T7.2 and T8.7 T7.2 Training and support of link workers in the RCT using the digital tool (M16-33) L: ACT Parallel to the link worker training in task T9.2 and the intervention delivery in T9.6 and T10.1, ACT will train and support link workers in using the digital tool. This task feeds also into T7.3. T7.3 Refine digital tool (M23-35) L: ACT Based on the experiences from T7.2, ACT will refine the digital tool. T7.3 feeds into T7.4. T7.4 Prepare the publication of the digital tool in an open repository (M31-36) L: ACT As final task in this WP, ACT will prepare the publication of the digital tool in an open source software repository. This will include a thorough documentation of the tool. The publication will be in close alliance with the coordination and data management (WP2) and communication and dissemination (WP17) to be in line with the data management incl. FAIR principles as well as to reach the potential users of the tool. This leads to D7.1&D7.2.

Work package WP8 – RCT Preparation incl. Ethics

Work Package Number	WP8	Lead Beneficiary	1 - Charite
Work Package Name	RCT Preparation incl. Ethics		
Start Month	1	End Month	20

Objectives
To prepare all necessary documents, files and trainings and gain all regulatory approvals to conduct the multinational pragmatic RCT (T8.1-T8.10) This WP contributes mainly to Obj.2, KPI2.1

Description
L: Charité; P: UoP, UOXF and all RCT hubs T8.1 Plan and conduct a joint workshop on SP RCTs – lessons learned (M1-6) L: Charité; P: all RCT hubs Charité will plan an online workshop together with the implementation board and advisory board on lessons learned from previously conducted RCTs on SP. The workshop will present different RCT approaches, e.g., from Ireland, Sweden, and Germany. We will discuss together what the lessons learned are and how they might influence aspects of the RCT implementation in SP-EU. The results of T8.1 will feed especially into T8.2. T8.2 Develop study protocol (M1-12) L: Charité; P: all RCT hubs The study protocol will be developed by the team at Charité in close exchange with all RCT hubs. The adaptation from WP5 (Co-Creation) will be incorporated in the study protocol. The study protocol will be published open publicly available before start of the study. The results of T8.2 feed then into T8.4 and T8.5 T8.3 Develop a data security and data sharing plan for the trial (M1-6) L: Charité; P: all RCT hubs The data security and data sharing plan will be developed by the team at Charité in close exchange with all RCT hubs.

The data security and data sharing plan will be closely harmonized with the overall data management plan (T1.4). The EB will appoint a Data Safety and Management Board (DSMB). The results of T8.4 feed into T8.5 and T8.6.

T8.4 Write and obtain ethics and regulatory approval for the lead hub at Charité (M7-14) L: Charité
This task will be conducted by the team at Charité. In SP-EU, we will first obtain ethics approval for the lead hub at Charité and this approval will be used by all other RCT hubs. Based on the study protocol, we will prepare all study documents including conformed consent. To accelerate the first submission, we will use the preliminary adapted SP intervention from T5.4 and add potential changes from the consensus conference in T5.5 as an amendment. The approval by the lead ethics committee at Charité will be the third milestone of SP-EU (MS3). T8.4 feeds into T8.5 and is prerequisite for WP9.

T8.5 Write and obtain ethics and regulatory approval for all other hubs (M10-20) L: Charité; P: all RCT hubs
This task will be conducted by the teams of each RCT hub individually closely supported by the team at Charité. Based on the documents from Charité, each RCT hub will prepare their ethics documents including informed consent material for patients. T8.5 is prerequisite for the conduction of WP9 in the different RCT hubs.

T8.6 Prepare all study materials including Trial Master File (TMF), Hub Investigator File (HIF) and Site Investigator File (SIF) (M7-14) L: Charité; P: all RCT hubs
The team at Charité will prepare all study material in close exchange with all other RCT hubs. This encompasses especially the TMF and SIF and all further necessary study documentation not covered in T8.2, T8.3 and T8.4. The results of T8.6 feed into WP9.

T8.7 Designing training of study nurses, link workers and study sites (M11-16) L: UOXF; P: UoP, Charité
Incorporating the results of WP5, we will develop the training of study nurses, link workers and study sites based on already existing training and research materials (cf. T5.1). The training will be prepared as a blended learning training including written and prerecorded video training material for asynchronous self-learning, synchronous online training sessions, and facilitated online peer-led discussion. All training manuals and materials will be made public via the knowledge hub on the webpage. T8.7 feeds into T9.2.

T8.8 Recruiting study sites (M9-19) L: Charité; P: all RCT hubs
This task will be conducted by the teams of each RCT hub individually closely supported by the team at Charité. At each RCT hub, we aim to recruit 5 to 10 GP practices as study sites depending on the size of GP practices in the given country. These sites shall be chosen in way, that the recruitment goal of each RCT hub can be achieved in a recruitment period of 6 months. The RCT hubs will use their local research practice networks (if available) and otherwise the teaching networks of the according university unit. The researchers of the RCT hubs will visit each of the potential sites to introduce the study and check for the fit of the site for the study. The recruitment of the study sites is prerequisite for T9.3.

T8.9 Writing statistical analysis plan (SAP) (M11-16) L: Charité
The biometry unit of the Charité team will write the SAP. The SAP shall be finished before the first patient is recruited into the study. The SAP will describe all necessary steps for the analysis of the primary endpoint and the secondary endpoints of the study. It will be used in T11.1.

T8.10 Programming electronic Case Report Form (eCRF) (M11-16) L: Charité
This task will be conducted by the Clinical Trial Office (CTO) at Berlin in close cooperation with the sponsor team at Charité which is in exchange with all RCT hubs. The CTO will program the web-based data capture (eCRF) on the secuTrial platform which secures a high standard of data security and enables efficient monitoring of a study with many study sites. The study database will be hosted in a secured data centre of the Charité by Charité GB IT and will be backed up periodically. The eCRF will be tested and validated within the CTO DM department and the study team. Dummy data sets will be prepared for statistical programming in T11.1. Furthermore, the database and the eCRF can be adapted due to changes in the study design, if the study is not released into productive state. A training on using the eCRF will be made ready for the study nurses. The eCRF from this task will feed into T9.1, T9.4 and T9.5.

Work package WP9 – RCT Implementation (1/2)

Work Package Number	WP9	**Lead Beneficiary**	1 - Charite
Work Package Name	RCT Implementation (1/2)		
Start Month	16	**End Month**	30

Objectives
To conduct the first half of the multinational pragmatic RCT (T9.1-T9.11). Specific objective of this first of two WPs in trial implementation is to recruit the necessary number of patients into the RCT (T9.4). This WP contributes to Obj.2, KPI2.2

Description
L: Charité; P: all RCT hubs T9.1 RCT hub initiation (M16-21) L: Charité; P: all RCT hubs The RCT hub initiation will be conducted by the team at Charité. As soon as the ethical approval for the according RCT hub is ready, the team from Charité will visit each RCT hub. The Charité team will bring the TMF and all necessary material to the RCT hub. During the RCT hub initiation, the local team and the Charité team will talk through all details of the RCT implementation including monitoring. The researcher and the study nurse will be trained based on the training developed in T8.7 and T8.10. The RCT hub will receive the logins for the eCRF. The RCT hub will get a train-the-trainer program to initiate and train the study sites later on in T9.3 The plan for T9.1 is to initiate each RCT hub as early as possible; we plan to initiate one RCT hub each week whenever possible. The RCT hub initiation in T9.1 is the necessary precondition for each hub to proceed to T9.3ff. T9.2 Training of link workers (M16-22) L:UOXF; P: UoP, all RCT Hubs The training of link workers is led by the teams in UoP and UOXF in close collaboration with the team at Charité and the local RCT hubs. The training developed in T8.7 will be made available to the link workers one month before the according RCT hubs is going to recruit the first patient into the study. While the asynchronous content is ready from T8.7, synchronous training and peer-led discussion will be delivered in this task. As part of the training, the link workers will go out into the communities where the local study sites are located, get familiar with the communities and introduce themselves to supports and services in the community. They will be in contact with the local advisory boards to steer their community networking. Link Working will be inducted locally and liked to existing safeguarding in the community. T9.2 is prerequisite for the delivery of the intervention in T9.6 and T10.1. T9.3 Study site initiation (M16-24) L: Charité; P: all RCT hubs This task will be conducted by each RCT hub individually supervised by the Charité team. After successful RCT hub initiation in T9.1 the study sites recruited in T8.8 will be initiated by the local team consisting of a researcher, a study nurse and a link worker. They will visit each study site individually, approximately one to two visits per week, lasting half a day. The local hub team will train the study site personnel including GPs and nurses on awareness, recruitment of study participants and data entry into the eCRF. They will hand over a site investigator file (SIF) and informed consent material to each site. A successful study site initiation is the necessary precondition for recruitment of patients (T9.4). T9.4 Recruitment of patients (M17-30) L: Charité; P: all RCT hubs This task will be conducted by the team of the study site, usually a GP, supervised by the local RCT hub team. The practice team has to identify patients coming to the practice as eligible for the study. They have to give them all written informed consent information, answer all their question and obtain oral and written informed consent. The informed consent has to be archived securely in the SIF. The necessary patient data will be entered into the eCRF. To achieve the necessary sample size, each practice has to recruit 3 patients a month into the study over the course of 6 months. T9.4 feeds into T9.5. The first-patient-in is the fourth milestone of SP-EU (MS4). T9.5 Baseline data acquisition (M17-30) L: Charité; P: all RCT hubs This task will be conducted by each RCT hub team, supervised by the Charité team. After successful recruitment into the trial, the study nurse will get into contact with each patient. The study nurse will conduct a computer assisted data acquisition with each participant as soon as possible after the recruitment. The baseline data acquisition will be conducted primarily by phone, alternatively by video call or personal encounter. After baseline data acquisition, the patient will be randomized automatically into one of the two arms of the study. In case of the intervention arm, the patient will get an appointment with the local link worker. This task is prerequisite for T9.6/10.1 for those randomized into the intervention arm and for T9.7/T10.2 for all participants. T9.6 Intervention delivery (1/2) (M17-30) L: Charité; P: all RCT Hubs, UOXF, UoP This task will be conducted by the local RCT hub teams, supervised by the team at UoP together with the team at UOXF and at Charité. All participants randomized into the intervention arm will receive up to five consultations with the according link worker depending on the need. We assume that each participant will have at least one consultation, and every eighth participant will need all five consultations. This leads to an expectation of 2.6 consultations per participant and 2 consultations every second week at each site. The consultations will be conducted primarily at the study sites and

alternatively at the hub, the participants' homes or local community centres. The link worker will use the digital tool developed in WP6 during the consultations. The link worker will consistently be present in the neighbourhoods and communities and in close contact to the services and supports there. They will be supervised regularly by the teams in UoP and UOXF. We propose a four-level approach to supervision provided by: (i) GPs in local practice, who provide safety net and emergency case support at all times; (ii) hub researchers, who can provide more general support for the link workers at all times, including wellbeing, advice and guidance for their own health; (iii) the training team, who can provide ongoing guidance against the role description and delivery; (iv) and peer support mechanisms to allow free conversation, discussion and issues to feed up to other elements in this list.

T9.7 Six-months-follow-up data acquisition (1/2) (M23-30) L: Charité; P: all RCT hubs
This task will be conducted by each RCT hub team, supervised by the Charité team. Six months after baseline data acquisition, the study nurse will get into contact with each patient. The study nurse will conduct a computer assisted data acquisition with each participant parallel to T9.5.

T9.8 Twelve-months-follow-up data acquisition (1/2) (M29-30) L: Charité; P: all RCT hubs
This task will be conducted by each RCT hub team, supervised by the Charité team. Twelve months after baseline data acquisition, the study nurse will get into contact with each patient. The study nurse will conduct a computer assisted data acquisition with each participant parallel to T9.5 and T9.7.

T9.9 eCRF support (M17-30) L: Charité; P: all RCT hubs
This task will be conducted by the Clinical Trial Office (CTO) at Charité in close cooperation with the sponsor team which is in exchange with all RCT hubs. The CTO will ensure continuous access to the eCRF on secuTrial.and technical support for the RCT hubs during the study duration (administration of logins, roles, and rights). The CTO will conduct sequential data management to prepare final data cleaning in T11.2, T11.4.

T9.10 Study site monitoring (M17-30) L: Charité; P: all RCT hubs
This task will be conducted by each RCT hub individually supervised by the Charité team. Monitoring will be conducted after successful site initiation in T9.3 and after start of the recruitment of patients in the according site (T9.4). The hub team will visit the study site at least once for data monitoring. The hub team will check all informed consent forms, if they are correctly filled out, if all inclusion and exclusion criteria are met and if the documented data in the eCRF are correct. Depending on the result of this first early site visit, the hub team will decide based on an individual risk assessment if any further site visits shall be planned or if the final study site closing is sufficient.

T9.11 Study hub monitoring (M17-30) L: Charité; P: all RCT hubs
The RCT hub monitoring will be conducted by the team at Charité. Monitoring will be conducted after successful hub initiation in T9.1 and after start of the recruitment of patients in at least two sites belonging to the hub (T9.4). The Charité team will visit the RCT hub at least once for monitoring. The Charité team will check all documentation in the eCRF and have a close talk with all members of the hub team. Depending on the result of this first hub visit, the Charité team will decide based on an individual risk assessment if any further hub visits shall be planned or if the final RCT hub closing is sufficient.

Work package WP10 – RCT Implementation (2/2)

Work Package Number	WP10	**Lead Beneficiary**	1 - Charite
Work Package Name	RCT Implementation (2/2)		
Start Month	31	**End Month**	43

Objectives
To conduct the second half of the multinational pragmatic RCT (T10.1-T10.6). Specific objective of this second of two WPs in trial implementation is to finish the follow up data acquisition for the necessary number of participants (T10.2, T10.3). This WP contributes to Obj.2, KPI2.3

Description
L: Charité; P: all RCT hubs, UoP, UOXF / WP10 continues WP9 (split due to lump sum funding)

T10.1 Intervention delivery (2/2) (M31-33) continues T9.6

T10.2 Six-months-follow-up data acquisition (2/2) (M31-36) continues T9.7

T10.3 Twelve-months-follow-up data acquisition (2/2) (M31-42) continues T9.8;

T10.3 completes with last-patient-out as the sixth milestone of SP-EU (MS6)

T10.4 eCRF support (M31-43) continues T9.9

T10.5 Study site monitoring and closing (M31-39) L: Charité; P: all RCT hubs
This task will be conducted by each RCT hub individually supervised by the Charité team. Study site closing will be conducted after successful end of recruitment and intervention delivery in the according site (T9.4, T9.6, T10.1). The hub team will visit the study site for data monitoring. The hub team will check all informed consent forms, if they are correctly filled out, if all inclusion and exclusion criteria are met and if the documented data in the eCRF are correct. The hub team will discuss with the study site team all aspects of conducting the study on this site and document all mentioned aspects. Finally, the RCT hub team will close the study site.

T10.6 Study hub monitoring and closing (M35-43) L: Charité; P: all RCT hubs
The RCT hub closing will be conducted by the team at Charité. Study hub closing will be conducted after the lastpatient-out of this RCT hub, when the twelve-months-follow-up data acquisition has been finished (T10.3). The Charité team will visit the RCT hub a last time for monitoring. The Charité team will check all documentation in the eCRF and have a close talk with all members of the hub team. Finally, the Charité team will close the RCT hub.

Work package WP11 – RCT Analysis

Work Package Number	WP11	**Lead Beneficiary**	1 - Charite
Work Package Name	RCT Analysis		
Start Month	31	**End Month**	60

Objectives
To analyse primary and secondary endpoints of the RCT (T11.1-T11.6). This WP contributes to Obj.2, KPI2.3, KPI2.4, KPI2.5, KPI2.7

Description

L: Charité

This work package will be conducted by the biometry team at Charité with support by the CTO and sponsor team at Charité.

T11.1 Programming statistical analysis (M31-36) L: Charité
This task will be conducted by the biometry team at Charité. This task is based on the SAP (T8.9) and the training data set provided by the CTO (T8.10). Prior to receiving the data, the biometry team will program the statistical analysis plan into an R script to be used in the analysis of primary and secondary endpoints in T11.3 and T11.5.

T11.2 Data cleaning six-months-follow-up (M37-39) L: Charité
This task will be conducted by the Clinical Trial Office (CTO) at Charité in close collaboration with all RCT hubs. Data acquired at baseline (T9.5) and in the six-month-follow-up (T9.7, T10.2) will be cleaned. For this purpose, the data will be exported from the database. The final data management process contains all plausibility, consistency and range checks of the data, described the data validation plan. The CTO team will ask the study hubs to answer all open queries. After data clarification processes are finished, the cleaned data will be available for the statistical analysis of the primary endpoint of the study. This task is prerequisite for T11.3.

T11.3 Data analysis primary endpoint (M40-45) L: Charité
This task will be conducted by the biometry team at Charité. This task is based on the SAP (T8.9) which has been programmed in R (T11.1). This programmed SAP will be applied on the cleaned data from T11.2. The biometry team

will analyse the primary endpoint and process all data in close collaboration with the sponsor team from Charité. The analysed primary endpoint is the seventh milestone of SP-EU (MS7) and prerequisite for WP15 and WP19.

T11.4 Data cleaning twelve-months-follow-up (M43-45) L: Charité
This task will be conducted by the Clinical Trial Office (CTO) at Charité in close collaboration with all RCT hubs. Data acquired in the twelve-month-follow-up (T9.8, T10.3) will be cleaned. For this purpose, the data will be exported from the database. The final data management process contains all plausibility, consistency and range checks of the data, described the data validation plan. The CTO team will ask the study hubs to answer all open queries. After data clarification processes are finished, the cleaned data will be available for the statistical analysis of the primary endpoint of the study. This task is prerequisite for T11.5.

T11.5 Data analysis secondary endpoints and secondary analyses (M46-60) L: Charité
This task will be conducted by the biometry team at Charité. This task is based on the SAP (T8.9) which has been programmed in R (T11.1). This programmed SAP will be applied on the cleaned data from T11.2 and T11.4. The biometry team will analyse the secondary endpoints and conduct sensitivity analyses. All data will be made available for all researchers involved in the consortium. This task feeds into WP15 and WP19.

T11.6 Preparing annotated trial data for embargoed publication in an open data repository (M54-60) L: Charité
This task will be conducted by the biometry team at Charité in close collaboration with the sponsor team at Charité. The trial data will be processed for publication in an open data repository. All trial data will be published as open data. All open data will be embargoed for two years after the end of SP-EU to allow for original scientific publications of the researchers in the consortium.

Work package WP12 – RCT Process Evaluation

Work Package Number	WP12	**Lead Beneficiary**	4 - UoP
Work Package Name	RCT Process Evaluation		
Start Month	19	**End Month**	48

Objectives
To assess the fidelity and quality of the trial implementation (T12.1-T12.4). This WP contributes to Obj.2

Description
L: UoP; P: UOXF, UBERN T12.1 Conduct qualitative interviews with stakeholders involved in the trial (M19-36) L: UoP; P: UOXF, UBERN The task will build on and be informed by WP14. The researchers will conduct qualitative interviews with stakeholders involved in the trial either in person or via video call. The number, breadth, and sampling for interviews will be informed by data collected in WP 14, and through consultation with recruited link workers. The interviews will be transcribed verbatim and anonymized afterwards. Stakeholders to be interviewed are patients, GPs, link worker, study nurses and supports and services in the community. The interviews will be on the implementation of the trial and potential barriers which might have come up as well as potential breaches of fidelity. T12.1 feeds directly into T12.2. T12.2 Qualitative analysis of stakeholder interviews (M29-41) L: UoP; P: UOXF, UBERN The transcribed interviews from T12.1 will be analysed jointly by the teams using framework analysis using the conceptual models described below (T12.4) to develop consistent descriptions of the determinants (barriers and facilitators), actions undertaken (implementation strategies), and aims/objectives of the pathway (outcomes) to produce site level implementation logic models where useful. This task feeds into T12.4. T12.3 Analysis of quantitative process data (M31-43) L: UoP; P: UOXF, UBERN This task will be conducted by the teams in UoP and UOXF supported by the sponsor team and biometry team at Charité. Quantitative process data from the digital support tool as well as from the link worker documentation and from the eCRF will be given to process evaluation team at a anonymized way. The data will be analysed descriptively to assess the fidelity of the trial. This tasks feeds into T12.4

T12.4 Triangulation of qualitative and quantitative process evaluation (M44-48) L: UoP; P: UOXF, UBERN
The qualitative and quantitative analysis from T12.2 and T12.3 will be triangulated. We tabulate data using a bespoke matrix where the core components of SP are plotted against the core CFIR1.0 domains, allowing us to target further data collection, interpret results at a high-level and synthesize against known areas of interest. This triangulation will lead to an assessment of fidelity and quality of the trial. The results will help in evaluating the trial results and this feed into WP11. The results will also feed into writing a research policy document on spoke and hub health care interventions RCTs in primary care in Europe. This task will be conducted by the Clinical Trial Office (CTO) at Charité in close collaboration with all RCT hubs. All data acquired in the twelve-monthfollow-up (T10.2) will be cleaned. The CTO team will ask the study hubs to answer all open queries. This task is prerequisite for T11.5.

Work package WP13 – Health Economics Assessment

Work Package Number	WP13	**Lead Beneficiary**	9 - BUW
Work Package Name	Health Economics Assessment		
Start Month	5	**End Month**	60

Objectives
To assess the health economic impacts of SP for people in vulnerable situations (T13.1-T13.4). This WP contributes to Obj.2, KPI2.6 and Obj.4, KPI4.4

Description

L: BUW

T13.1 Develop a theory of change (M5-12) L: BUW
This task will be conducted by the team at BUW in close exchange with the EuroHealthNet-Team and all RCT hubs. Up to three online-workshops within the SP-EU project team will be conducted to model a preliminary theory of change. During the workshops: 1) the impact, outputs, long-term outcomes, intermediate and short-term outcomes of SP-EU will be visualized; 2) activities that could lead to the outcomes and outputs will be mapped and assumption critical for the success of the implementation will be made explicit. Supplemented by evidence from existing literature on social prescribing, the preliminary theory of change will be integrated into the discussions with local advisory boards in all hubs (T5.4) and into the consensus conference (T5.5) for validation purposes. The result of T13.1 feeds directly into T13.2, T13.3 (exploration of implementation costs) and T13.4.

T13.2 Support implementation of the intervention (M13-39) L: BUW
This task will be conducted by the team at BUW in close exchange with the RCT hubs, the sponsor team and the UoP and UOXF. The team at BUW will support the partners directly involved in delivering the intervention to implement this intervention successfully. This will also allow for a detailed documentation of implementation costs (see T.13.3). To deliver implementation the team will rely on the itfits toolkit, which was developed within the EU-project ImpleMentAll. The team will present the application of the toolkit to the partners and offer regular consultation hours to deal with the toolkit and help with questions on implementation. In addition, recommendations on implementations activities will be derived from the theory of change on a more general level (not context/hub/site-specific). This task helps with T9.6 and T10.1.

T13.3 Conduct health economic analysis of trial results (M40-50) L: BUW
This task will be conducted by the team at BUW in close exchange with the biometry team at Charité. Economic aspects of SP will be assessed using cost-consequence analysis (CCA), offering a flexible and multifaceted assessment of costs and outcomes. CCA will include the following cost components and resources utilized: a) provision costs of the intervention, b) running costs of the intervention at the different trial sites, c) implementation costs, d) resource use within the healthcare sector, e) resource use outside the health sector (costs of care packages that SP beneficiaries are in receipt of). Outcomes will be measured using the primary and secondary outcomes of the RCT. This task will use data from multiple sources and standardized approaches. Cost components a), b) and c) will be measured by applying a time-driven activity-based costing (TDABC) approach. Resource use within and outside the healthcare sector will be covered by the eCRF (T8.10). The cleaned trial data will be provided by T11.2 and T11.4. This tasks feeds into T13.4 and into WP 19.

T13.4 Develop a sustainable financing framework (M47-60) L: BUW
This task will be conducted by the team at BUW in collaboration with the EuroHealthNet team. The primary objective is to provide a framework presenting the overall picture of how the intervention was implemented and at which costs, what contextual factors affected the results and for whom the intervention (and implementation) was efficient and through what mechanisms. Further a holistic view will be generated on how benefits and costs are distributed between stakeholders. Methodologically we will rely on realist methodology, the method of socioeconomic impact assessment and the policy cost-effectiveness as conceptual and methodological starting points for development.

Work package WP14 – Comparative Qualitative Study

Work Package Number	WP14	**Lead Beneficiary**	10 - UOXF
Work Package Name	Comparative Qualitative Study		
Start Month	13	**End Month**	36

Objectives

To explore enabling and limiting factors to implement SP as a solution to improve access to health and care services for people in vulnerable situations as a scalable and transferable solution for different European countries. (T14.1-T14.4). This WP contributes to Obj.3, KPI3.1, KPI 3.2.

Description

L: UOXF; P: UoP, BUW, WMU, GÖG, UNL

T14.1 Preparation of data acquisition incl. ethics (M13-18) L:UOXF; P: UoP, BUW, WMU, GÖG, UNL
This task will be conducted by all five teams and led by the team at UOXF. An initial in-person meeting for the team involved in this work package will be tied into the second general assembly meeting. This is important for establishing a good working relationship between different members of this groups and ensuring they are working in a similar manner. The teams will develop an interview topic guide, which will be informed by the co-creation work package. It will be translated into the relevant languages. Study material will be prepared and translated. The teams will write and publish a study protocol and will seek ethics approval first in the leading partner UOXF and then in all partners involved. This task is a prerequisite for T14.2.

T14.2 Conduct qualitative stakeholder interviews (M19-23) L:UOXF; P: UoP, BUW, WMU, GÖG, UNL
Based on the interview guide and study protocol developed in T14.1 the teams will conduct interviews with 20 stakeholders in each of the countries. The stakeholders will be sampled purposively to include experts and stakeholders involved in different parts of social prescribing – patient representatives (at least one patient representative from each of the groups focused on in the research - LGBTIQ persons, refugees and first-generation immigrants, older adults living alone), healthcare professionals (particularly GPs) and voluntary-community sector representatives. In addition, system-level decision-makers (e.g. in healthcare) will be interviewed to understand issues related to implementation and sustainability of social prescribing in each country. Interviews will, primarily, be conducted remotely (using Teams or Zoom or telephone). This approach has been used successfully by the work package lead in other social prescribing research. All interviews will be audio-recorded and transcribed verbatim. Language translation will be arranged for interviewees when necessary, though the preference will be for interviews conducted in local language. The interview recordings and transcripts will be stored in a safe shared online storage space. Consent forms and background information on interviewees will be collected and stored safely online. 20 stakeholder interviews will be conducted in each country (100 in total). To reach this goal in T14.2 is the fifth milestone of SP-EU (M5). T14.2 feeds into T14.3 and T14.4.

T14.3 Country specific qualitative data analysis (M20-28) L:UOXF; P: UoP, BUW, WMU, GÖG, UNL
This task will be conducted by each team (UoP, BUW, WMU, GÖG, UNL) separately while the team at UOXF will supervise this task to keep it synchronized over all countries. Framework analysis will be used to analyse transcripts produced in T14.2. The computer programme MAXQDA will be used to allow for sharing of coding and analysis across countries. This task feeds into T14.4.

T14.4 Joint comparative qualitative data analysis (M29-36) L:UOXF; P: UoP, BUW, WMU, GÖG, UNLThis task will be conducted with all five teams, led by UOXF. It builds on the results of T14.3. The teams will meet online and in person bimonthly throughout this work package to discuss data. As part of framework analysis, a single thematic framework

will be developed and used across countries to produce charts that summarize data from across countries. This task will lead to a preprint article that compares and contrasts qualitative data from across these countries to assess enabling and limiting factors to SP (KPI 3.2).

Work package WP15 – Co-Interpretation of Results

Work Package Number	WP15	Lead Beneficiary	7 - UNL
Work Package Name	Co-Interpretation of Results		
Start Month	49	End Month	60

Objectives
To interpret the results of trial and Comparative Qualitative Study jointly with representatives of people in vulnerable situations and health care providers (T15.1-T15.3). This WP contributes to Obj.1, KPI1.3, KPI1.4, KPI1.5

Description
L: UNL; P: AAU, UBERN, Charité, WONCA Europe, SB, DSIH, SRK-Bern T15.1 Prepare co-interpretation workshops (M49-50) L: UNL; P: AAU, UBERN, Charité, WONCA Europe, SB, DSIH, SRK-Bern, This task will be conducted by the teams in UNL, AAU, Charité and UBERN and coordinated by the team from UNL. In this task the results from WPs 11, 12, 13 and 14 will be processed into easy understandable presentations tailored for four target groups: Representatives of a) LGBTIQ persons, b) refugees and first-generation immigrants and c) older adults living alone and d) general practitioners. These presentations will involve all necessary material to understand results, figures and numbers. This task is a prerequisite for T15.2. T15.2 Conduct co-interpretation workshops with each group (M51-54) L: UNL; P: AAU, UBERN, Charité, WONCA Europe, SB, DSIH, SRK-Bern This task is based on the material produced in T15.1. We will conduct a workshop with each of the representatives of people in vulnerable situations and one workshop with GPs as health care providers. In these workshops, we will present the results of the different parts of SP-EU and discuss them with the participants. We will focus on two major aspects a) how to interpret the results and b) which conclusions to draw from the results. The outcomes of the workshops will be visualized and the participants will receive these. T15.2 feeds into T15.3. T15.3 Assemble co-interpretations into a document tailored for the according groups (M55-60) L: AAU; P: UNL, UBERN, Charité, WONCA Europe, SB, DSIH, SRK-Bern, We will assemble all materials produced in the co-interpretation workshops into documents tailored for each of the groups, so they can be used for dissemination and exploitation in WP19

Work package WP16 – Dissemination, Exploitation, Communication & Policy (1/4)

Work Package Number	WP16	Lead Beneficiary	17 - NASP
Work Package Name	Dissemination, Exploitation, Communication & Policy (1/4)		
Start Month	1	End Month	12

Objectives
To prepare communication and dissemination over the course of the whole project and make SP-EU known with stakeholders (T16.1-T16.7). This WP contributes to Obj.4, KPI4.1a and Obj.5., KPI5.1, KPI5.2

Description
L: NASP; P: EuroHealthNet, concentris T16.1 Identify target groups for communication and dissemination (M1-3) L: NASP; P: EuroHeatlhNet and all partners NASP will carry out a mapping exercise to draw up list of target groups & stakeholders such as service providers, health and care workers, researchers, scholars and scientists, policy-makers, EU level actors (European Commission and the newly elected European Parliament) and Brussels-based networks. At national and regional levels, we will additionally engage with health authorities to identify relevant actors in each setting. EuroHealthNet will also contribute to engaging with the scientific/research community, specifically those engaged in their Partnership and those who they are in contact with via EU-funded projects. A database with all target groups and stakeholders including contact persons whenever possible will be made available for all partners in the project. This database will be updated over the course of the project. This task is a prerequisite for T16.5 and T16.7. T16.2 Develop a corporate identity and branding of the project (M1-4) L: concentris This task will be conducted by the concentris team at close collaboration with the NASP team and EuroHealthNet team as well as the coordinator being in exchange with all partners. Based on the preliminary branding during the preparation of this proposal, we will develop a corporate identity of SP-EU (incl. a project logo) to be used throughout the project and beyond. This task feeds into T16.3, T16.4 and T16.5. T16.3 Design and program an appealing project website including knowledge hub (M1-5) L: concentris This task will be conducted by the concentris team in close collaboration with NASP, EuroHealthNet and the coordinator team. We will program a website, including a password-protected intranet, which will inform patients, stakeholders, health care professionals and policy makers on the project in general, which will post news of the project and includes a knowledge hub, where all knowledge produced by SP-EU will be made available to the public. The website will be updated and maintained during the project. This task feeds into T17.1, T18.1, T19.1. T16.4 Create a communication and dissemination tool kit (M1-6) L: concentris This task will be conducted by the concentris team in close collaboration with NASP, EuroHealthNet and the coordinator team. Based on the corporate identity (T16.2), we will develop templates for brochures, presentations, flyers, and handouts. These can be used by all partners throughout the project. This task feeds into T16.5 and T16.7. T16.5 Write an initial communication and dissemination plan (M1-6) L: NASP; P: EuroHealthNet, concentris This task will be conducted by the NASP team in close collaboration with EuroHealthNet supported by the PMO. We will develop a joint communication and dissemination plan bringing all results of T16.-T16.4 together in a package every partner in SP-EU can rely on. The communication and dissemination plan will include situation, objectives, strategy, tactics, actions and controls. We will carry out audience development for identified target groups and plan targeted communications outputs accordingly. We will also create a draft exploitation plan and deliver this across the life course of the project. T16.5 feeds into all further tasks of WP16-19. T16.6 Set up social media accounts (M4-10) L: NASP Based on audience profiles and needs, NASP will set up appropriate social media accounts in line with the communication and dissemination plan – current thinking is that these will be LinkedIn and X. NASP will manage these social media accounts and publicize findings on SP-EU. This task feeds into T16.6, T17.1, T18.1, T19.1. T16.7 Communicate regularly with stakeholders and target groups about the current status and findings of the project (M1-12) L: NASP; P: EuroHealthNet, concentris This continuing task is led by the NASP team in close collaboration with the EuroHealthNet and concentris and with input from all SP-EU partners. Based on the communication and dissemination plan (T16.5) We will use the stakeholders and groups identified in T16.1 and the channels and materials created in T16.2-16.3 and T16.6 to communicate and disseminate content and results of SP-EU. We will address stakeholders and target groups on national and European level. We will use different forms of communication materials from short clips to texts.

Work package WP17 – Dissemination, Exploitation, Communication & Policy (2/4)

Work Package Number	WP17	**Lead Beneficiary**	17 - NASP
Work Package Name	Dissemination, Exploitation, Communication & Policy (2/4)		
Start Month	13	**End Month**	30

Objectives
To make the project well known and interconnected with relevant stakeholders (T17.1-T17.2). This WP contributes to Obj.4 and Obj.5., KPI5.1, KPI5.2, KPI5.3

Description
L: NASP, P: EuroHealthNet, concentris / WP17 continues WP16 (split due to lump sum funding) T17.1 Communicate regularly with stakeholders and target groups about the current status and findings of the project (M13-30) continues T16.7 We will provide annual reports on the use of all communication channels such as website, social media, and classic press. NASP will evaluate the outcomes of the different measures and channels and make recommendations for future use based on these findings. T17.2 Engaging and linking up with related initiatives (mth13-18) L: EuroHealthNet; P: Charité Lead by EuroHealthNet, this task focuses on creating connections and synergies with multipliers in the field, EU channels and other relevant HE projects, or ongoing/future national projects across Europe. This task will maintain connections with identified projects and organisations (T16.1) to encourage synergies and reduce overlaps in planned work. A list of cooperation, dissemination and exploitation activity potential with other related EU projects will be created, including with all other projects funded in the same call (these could include joint communication activities, workshops, policy briefs, etc.), and regular meetings will be put in place (both online and in-person). Exploiting the consortium members' presence in several EU and WHO health related networks, we will continuously explore the horizon of relevant European and international initiatives and actively expand our presence in relevant actions.

Work package WP18 – Dissemination, Exploitation, Communication & Policy (3/4)

Work Package Number	WP18	Lead Beneficiary	15 - EuroHealthNet
Work Package Name	Dissemination, Exploitation, Communication & Policy (3/4)		
Start Month	31	End Month	48

Objectives
To disseminate first results and interact with stakeholders and target groups (T18.1-T18.6). This WP contributes to Obj.4, KPI4.2 and Obj.5., KPI5.2, KPI5.3, KPI5.4

Description
L: EuroHealthNet, P: NASP, concentris / WP18 continues WP17 (split due to lump sum funding) T18.1 Communicate regularly with stakeholders and target groups about the current status and findings of the project (M31-48) continues T17.1 T18.2 Develop and disseminate policy implications (M31-48) L: EuroHealthNet This task will be conducted by EuroHealthNet in close collaboration with BUW and all partners. The potential opportunities for SP-EU on policy are broad. This task will consist of monitoring EU mechanisms which could be used to advance SP while bringing together the project policy outcomes in a systematic way, through both internal work with partners and consultation with stakeholders. Policy messages will be developed and presented in policy briefs, factsheets, or other dissemination materials oriented towards policy- and decision-makers. Those messages will be communicated and disseminated in targeted ways such as bilateral meetings with relevant policymakers, national and EU policy roundtables, organization of/participation in workshops and events, inputs to relevant consultations, as well as specialized or general media channels. T18.3 SP-EU for governing authorities at national level (M38-48) L: EuroHealthNet This task will be conducted by EuroHealthNet in close collaboration with the partner from the specific country. We will conduct eight policy roundtables at national level in participating countries. The general purpose of the policy roundtables is to support Member States to advance the debate on social prescribing and achieve better action, in the most appropriate way for the different countries. Each country will define their specific objectives. The policy dialogue organization guide

and reporting template will be developed using CHRODIS PLUS Joint Action methodology for national level policy dialogues to support the national teams. We plan to conduct four roundtables in T18.3 and four roundtables in T19.2.

T18.4 Compile a toolkit to implement SP for service providers (M31-42) L: NASP
This task will be conducted by NASP in close collaboration with Charité and WONCA Europe and the implementation board. Based on the adaptation of SP for vulnerable groups, the training for link workers and GPs, and the digital linkworking tool and its documentation, we will compile a toolkit to implement SP which targets especially service providers. This toolkit will be evaluated by the implementation board and WONCA Europe as service provider representative. It will be made available for use on SP-EU's website and promoted via social media and via the existing networks on social prescribing. We will also make it available to GPs via a practical workshop at the WONCA Europe conference. Partners may promote it with workshops at national GP conferences.

T18.5 Creating an informative short film on the results of SP-EU (M43-48) L: NASP; P: EuroHealthNet
Based on the results of SP-EU, NASP will create a short film which explains the results in a lay language easily accessible for the general public. The film will be translated in different European languages to reach a broad audience. The film will be distributed mainly via social media channels of SP-EU and their partners with at least 1,000 views.

T18.6 Compiling educational resources to support the implementation of SP-EU (M45-48) L: NASP
This task will be conducted by NASP in close collaboration with Charité and WONCA Europe and the implementation board. Based on the toolkit from T18.4 and the film from T18.5, we will compile additional educational resources used during SP-EU and combine it with other available educational resources on SP. These will be made available alongside the toolkit and other briefings on SP-EU's website.

Work package WP19 – Dissemination, Exploitation, Communication & Policy (4/4)

Work Package Number	WP19	**Lead Beneficiary**	15 - EuroHealthNet
Work Package Name	Dissemination, Exploitation, Communication & Policy (4/4)		
Start Month	49	**End Month**	60

Objectives
To prepare the exploitation of the outcomes of SP-EU beyond the timeline of the project (T19.1-T19.5). This WP contributes to Obj.4, KPI4.1b, KPI4.2, KPI4.3, KPI4.4 and Obj.5., KPI5.3

Description
L: EuroHealthNet; P: NASP, concentris / WP19 continues WP18 (split due to lump sum funding) T19.1 Communicate regularly with stakeholders and target groups about the current status and findings of the project (M49-60) continues T18.1 T19.2 Compiling educational resources to support the implementation of SP-EU (M45-48) continues T18.6 T19.3 SP-EU for governing authorities at national level (M49-60) continues T18.3 T19.4 Bringing SP-EU at the EU level (M55-60) L: EuroHealthNet; P: NASP We will conduct one EU level policy dialogue to foster transition towards more sustainable health systems. The EU level Policy dialogue will raise awareness and offer tools for capacity building, including the use of European Union funding instruments, to adopt social prescribing as an effective way to promote health and prevent diseases in EU Member States. The main outcome of the policy dialogue should gather actionable recommendations for EU and national policy and decision-makers. T19.5 Disseminating and exploiting results and ensuring long-term impact and sustainability (M49-60) L: Eurohealthnet; P: NASP This task will focus on developing the final exploitation plan. All partners will be involved collaboratively. We will steer the dissemination of the project and take specific measures to ensure that SP-EU results are transferrable and exploited by users. It is fundamentally linked to all research and innovation tasks, particularly to those producing exploitable results. Dissemination will focus on publishing articles in open access journals as well as more informally in scientific

blogs and magazines and sharing results during scientific conferences. Exploitation activities will focus on providing policymakers with the tools that they need and ensuring they can use them in their everyday activities (e.g. sustainable financing framework). This will involve developing a policy brief (including recommendations), as well as organizing open door days in the hubs for EU, national and local policymakers, with all materials made publicly available on the project website. The exploitation plan will further detail activities going beyond the project timeline to ensure long-term sustainability and impact, which will focus on identifying additional authorities/countries to make use of key and developing actions to promote the uptake of SP-EU results amongst these stakeholders.

STAFF EFFORT

Staff effort per participant

Grant Preparation (Work packages - Effort screen) — Enter the info.

Participant	WP1	WP2	WP3	WP4	WP5	WP6	WP7	WP8	WP9	WP10	Total Person-Months
1 - Charite	12.00	18.00	18.00	12.00	7.20			30.25	34.75	24.00	194.40
2 - GÖG	0.10	0.10	0.10	0.10							14.90
3 - concentris	7.00	12.00	12.00	7.00							42.50
4 - UoP	0.10	0.10	0.10	0.10				0.50	0.50	0.25	23.85
5 - WMU	0.10	0.10	0.10	0.10	0.60			2.26	22.76	11.00	51.02
6 - IDIAPJGol	0.10	0.10	0.10	0.10				0.10	19.00	8.00	27.60
6.1 - ICS					0.60			2.25	3.75	3.00	9.60
7 - UNL	0.10	0.10	0.10	0.10	2.10			2.25	22.75	11.00	59.00
8 - AAU	0.10	0.10	0.10	0.10	7.20			2.25	22.75	11.00	49.70
9 - BUW	0.10	0.10	0.10	0.10							26.90
10 - UOXF	0.10	0.10	0.10	0.10				3.10	7.10	0.25	36.85
11 - UKE	0.10	0.10	0.10	0.10	0.60			2.25	22.75	11.00	37.10
12 - ACT	0.10	0.10	0.10	0.10		16.80	3.00				20.20
13 - DZZZ	0.10	0.10	0.10	0.10	0.60			2.25	22.75	11.00	37.10
14 - UKF	0.10	0.10	0.10	0.10	0.60			2.25	22.25	11.00	36.60
15 - EuroHealthNet											33.00
16 - UGent	0.10	0.10	0.10	0.10	0.60			2.25	22.75	11.00	37.10
17 - NASP	0.10	0.10	0.10	0.10							12.40
18 - WONCA Europe	0.10	0.10	0.10	0.10							0.40

Staff effort per participant

Grant Preparation (Work packages - Effort screen) — Enter the info.

Participant	WP1	WP2	WP3	WP4	WP5	WP6	WP7	WP8	WP9	WP10	Total Person-Months
19 - DSIH	0.10	0.10	0.10	0.10							0.40
20 - SB	0.10	0.10	0.10	0.10							0.40
21 - UBERN					6.00			2.00	24.00	12.00	85.00
22 - SRK-Bern					1.00						2.00
Total Person-Months	20.70	31.70	31.70	20.70	27.10	16.80	3.00	53.96	247.86	124.50	838.02

Staff effort per participant

Grant Preparation (Work packages - Effort screen) — Enter the info.

Participant	WP11	WP12	WP13	WP14	WP15	WP16	WP17	WP18	WP19	Total Person-Months
1 - Charite	32.00				6.00			0.10	0.10	194.40
2 - GÖG				14.40				0.10		14.90
3 - concentris						3.00	0.50	0.50	0.50	42.50
4 - UoP		7.70		14.40				0.10		23.85
5 - WMU				13.90				0.10		51.02
6 - IDIAPJGol								0.10		27.60
6.1 - ICS										9.60
7 - UNL				14.40	6.00			0.10		59.00
8 - AAU					6.00			0.10		49.70
9 - BUW			12.00	14.40				0.10		26.90
10 - UOXF		12.70		13.20				0.10		36.85
11 - UKE								0.10		37.10

Staff effort per participant

Grant Preparation (Work packages - Effort screen) — Enter the info.

Participant	WP11	WP12	WP13	WP14	WP15	WP16	WP17	WP18	WP19	Total Person-Months
12 - ACT										20.20
13 - DZZZ								0.10		37.10
14 - UKF								0.10		36.60
15 - EuroHealthNet						4.00	6.50	13.00	9.50	33.00
16 - UGent								0.10		37.10
17 - NASP						2.40	3.60	3.60	2.40	12.40
18 - WONCA Europe										0.40
19 - DSIH										0.40
20 - SB										0.40
21 - UBERN		36.00			5.00					85.00
22 - SRK-Bern					1.00					2.00
Total Person-Months	32.00	56.40	12.00	84.70	24.00	9.40	10.60	18.40	12.50	838.02

LIST OF DELIVERABLES

Deliverables

Grant Preparation (Deliverables screen) — Enter the info.

The labels used mean:

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Sensitive — limited under the conditions of the Grant Agreement
EU classified —RESTREINT-UE/EU-RESTRICTED, CONFIDENTIEL-UE/EU-CONFIDENTIAL, SECRET-UE/EU-SECRET under Decision 2015/444

Deliverable No	Deliverable Name	Work Package No	Lead Beneficiary	Type	Dissemination Level	Due Date (month)
D1.1	Rules of Procedure	WP1	1 - Charite	OTHER	PU - Public	3
D1.2	Data Management Plan - V1	WP1	1 - Charite	DMP — Data Management Plan	SEN - Sensitive	6
D2.1	Ethics Monitoring Report	WP2	1 - Charite	R — Document, report	PU - Public	18
D2.2	Early-Stage Career Support Report	WP2	1 - Charite	R — Document, report	PU - Public	24
D2.3	Data Management Plan - V2	WP2	1 - Charite	DMP — Data Management Plan	SEN - Sensitive	30
D3.1	Ethics Monitoring Update	WP3	1 - Charite	R — Document, report	PU - Public	36
D3.2	Gender and Diversity Report	WP3	1 - Charite	R — Document, report	PU - Public	42
D3.3	Data Management Plan - V3	WP3	1 - Charite	DMP — Data Management Plan	SEN - Sensitive	48
D4.1	Open Science Report	WP4	1 - Charite	R — Document, report	PU - Public	54
D4.2	Governance Report	WP4	3 - concentris	R — Document, report	SEN - Sensitive	60
D4.3	Data Management Plan - V4	WP4	1 - Charite	DMP — Data Management Plan	SEN - Sensitive	60
D5.1	Co-Creation Report	WP5	8 - AAU	R — Document, report	PU - Public	12

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Deliverable No	Deliverable Name	Work Package No	Lead Beneficiary	Type	Dissemination Level	Due Date (month)
D6.1	ALTAI-Report	WP6	12 - ACT	R — Document, report	PU - Public	6
D6.2	Prototype of Digital Tool	WP6	12 - ACT	DEM — Demonstrator, pilot, prototype	PU - Public	9
D7.1	Digital Tool Documentation	WP7	12 - ACT	OTHER	PU - Public	24
D7.2	Digital Tool	WP7	12 - ACT	OTHER	PU - Public	30
D8.1	Study Initiation Package	WP8	1 - Charite	R — Document, report	PU - Public	14
D8.2	GP Practice Recruitment Report	WP8	1 - Charite	R — Document, report	PU - Public	18
D9.1	Midterm Recruitment Report	WP9	1 - Charite	R — Document, report	PU - Public	23
D10.1	Final Follow-Up Report	WP10	1 - Charite	R — Document, report	PU - Public	36
D11.1	Report on the status of posting trial results	WP11	1 - Charite	R — Document, report	SEN - Sensitive	60
D11.2	Anonymized Trial Data	WP11	1 - Charite	DATA — data sets, microdata, etc	SEN - Sensitive	60
D12.1	Trial Fidelity Report	WP12	4 - UoP	R — Document, report	PU - Public	48
D13.1	Theory of Change Model	WP13	9 - BUW	OTHER	PU - Public	18
D13.2	Health Economic Publication Update	WP13	9 - BUW	R — Document, report	SEN - Sensitive	54
D14.1	Qualitative Recruitment Report	WP14	10 - UOXF	R — Document, report	PU - Public	23
D14.2	Qualitative Study Publication Update	WP14	10 - UOXF	R — Document, report	SEN - Sensitive	36

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Deliverable No	Deliverable Name	Work Package No	Lead Beneficiary	Type	Dissemination Level	Due Date (month)
D15.1	Co-Interpretation Report	WP15	7 - UNL	R — Document, report	PU - Public	58
D15.2	Co-Creation Publication Update	WP15	8 - AAU	R — Document, report	SEN - Sensitive	58
D16.1	Website incl. Knowledge Hub	WP16	3 - concentris	DEC —Websites, patent filings, videos, etc	PU - Public	5
D16.2	Communication and Dissemination Plan - V1	WP16	17 - NASP	R — Document, report	PU - Public	6
D17.1	Report on the Use of Communication Channels	WP17	17 - NASP	R — Document, report	PU - Public	27
D17.2	Communication and Dissemination Plan - V2	WP17	17 - NASP	R — Document, report	PU - Public	30
D18.1	Report on the Established Dissemination Networks	WP18	15 - EuroHealthNet	R — Document, report	PU - Public	39
D18.2	Explanatory Film on SP-EU for the Public	WP18	17 - NASP	DEC —Websites, patent filings, videos, etc	PU - Public	48
D18.3	Communication and Dissemination Plan - V3	WP18	15 - EuroHealthNet	R — Document, report	PU - Public	48
D19.1	Report on Policy Roundtables	WP19	15 - EuroHealthNet	R — Document, report	PU - Public	60
D19.2	Final Exploitation Plan	WP19	15 - EuroHealthNet	R — Document, report	PU - Public	60

Deliverables

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Deliverable No	Deliverable Name	Work Package No	Lead Beneficiary	Type	Dissemination Level	Due Date (month)
D19.3	Communication and Dissemination Plan - V4	WP19	15 - EuroHealthNet	R — Document, report	PU - Public	60

Deliverable D1.1 – Rules of Procedure

Deliverable Number	D1.1	**Lead Beneficiary**	1 - Charite
Deliverable Name	Rules of Procedure		
Type	OTHER	**Dissemination Level**	PU - Public
Due Date (month)	3	**Work Package No**	WP1

Description
Governance document including the rules of procedure for general assemblies, executive board, and all advisory boards as well as code of conduct for SP-EU

Deliverable D1.2 – Data Management Plan - V1

Deliverable Number	D1.2	**Lead Beneficiary**	1 - Charite
Deliverable Name	Data Management Plan - V1		
Type	DMP — Data Management Plan	**Dissemination Level**	SEN - Sensitive
Due Date (month)	6	**Work Package No**	WP1

Description
The initial data management plan for SP-EU

Deliverable D2.1 – Ethics Monitoring Report

Deliverable Number	D2.1	**Lead Beneficiary**	1 - Charite
Deliverable Name	Ethics Monitoring Report		
Type	R — Document, report	**Dissemination Level**	PU - Public
Due Date (month)	18	**Work Package No**	WP2

Description
Report on how ethics has been applied in all parts of SP-EU, which ethical approvals have been sought and if there are any ethical problems and how they have been mitigated

Deliverable D2.2 – Early-Stage Career Support Report

Deliverable Number	D2.2	**Lead Beneficiary**	1 - Charite
Deliverable Name	Early-Stage Career Support Report		
Type	R — Document, report	**Dissemination Level**	PU - Public
Due Date (month)	24	**Work Package No**	WP2

Description
Report on support for early-stage career researchers in SP-EU

Deliverable D2.3 – Data Management Plan - V2

Deliverable Number	D2.3	**Lead Beneficiary**	1 - Charite
Deliverable Name	Data Management Plan - V2		
Type	DMP — Data Management Plan	**Dissemination Level**	SEN - Sensitive
Due Date (month)	30	**Work Package No**	WP2

Description
Data management plan updated for the second reporting period.

Deliverable D3.1 – Ethics Monitoring Update

Deliverable Number	D3.1	**Lead Beneficiary**	1 - Charite
Deliverable Name	Ethics Monitoring Update		
Type	R — Document, report	**Dissemination Level**	PU - Public
Due Date (month)	36	**Work Package No**	WP3

Description
Update on the ethics monitoring report if there are any ethical problems and how they have been mitigated

Deliverable D3.2 – Gender and Diversity Report

Deliverable Number	D3.2	**Lead Beneficiary**	1 - Charite
Deliverable Name	Gender and Diversity Report		
Type	R — Document, report	**Dissemination Level**	PU - Public
Due Date (month)	42	**Work Package No**	WP3

Description
Report on how gender and diversity are represented in consortium and project and what measures were taken to improve gender and diversity balance

Deliverable D3.3 – Data Management Plan - V3

Deliverable Number	D3.3	**Lead Beneficiary**	1 - Charite
Deliverable Name	Data Management Plan - V3		
Type	DMP — Data Management Plan	**Dissemination Level**	SEN - Sensitive
Due Date (month)	48	**Work Package No**	WP3

Description
Data management plan updated for the third reporting period.

Deliverable D4.1 – Open Science Report

Deliverable Number	D4.1	**Lead Beneficiary**	1 - Charite
Deliverable Name	Open Science Report		
Type	R — Document, report	**Dissemination Level**	PU - Public
Due Date (month)	54	**Work Package No**	WP4

Description
Report on the implementation of open science practices throughout the project and which data has been made publicly available

Deliverable D4.2 – Governance Report

Deliverable Number	D4.2	**Lead Beneficiary**	3 - concentris
Deliverable Name	Governance Report		
Type	R — Document, report	**Dissemination Level**	SEN - Sensitive
Due Date (month)	60	**Work Package No**	WP4

Description
Report with all minutes of general assemblies conducted

Deliverable D4.3 – Data Management Plan - V4

Deliverable Number	D4.3	**Lead Beneficiary**	1 - Charite
Deliverable Name	Data Management Plan - V4		
Type	DMP — Data Management Plan	**Dissemination Level**	SEN - Sensitive
Due Date (month)	60	**Work Package No**	WP4

Description
Data management plan updated for the fourth reporting period.

Deliverable D5.1 – Co-Creation Report

Deliverable Number	D5.1	**Lead Beneficiary**	8 - AAU
Deliverable Name	Co-Creation Report		
Type	R — Document, report	**Dissemination Level**	PU - Public
Due Date (month)	12	**Work Package No**	WP5

Description
Report of the co-creation process to adapt the SP intervention to the needs of vulnerable groups

Deliverable D6.1 – ALTAI-Report

Deliverable Number	D6.1	**Lead Beneficiary**	12 - ACT
Deliverable Name	ALTAI-Report		
Type	R — Document, report	**Dissemination Level**	PU - Public
Due Date (month)	6	**Work Package No**	WP6

Description
Assessment of the digital linkworking tool by using the Assessment List for Trustworthy Artificial Intelligence including all measures taken for identified risks

Deliverable D6.2 – Prototype of Digital Tool

Deliverable Number	D6.2	**Lead Beneficiary**	12 - ACT
Deliverable Name	Prototype of Digital Tool		
Type	DEM — Demonstrator, pilot, prototype	**Dissemination Level**	PU - Public
Due Date (month)	9	**Work Package No**	WP6

Description
Prototype of digital linkworking tool for feedback by hubs and advisory boards

Deliverable D7.1 – Digital Tool Documentation

Deliverable Number	D7.1	**Lead Beneficiary**	12 - ACT
Deliverable Name	Digital Tool Documentation		
Type	OTHER	**Dissemination Level**	PU - Public
Due Date (month)	24	**Work Package No**	WP7

Description
Documentation of the digital tool developed to make the use as open source software widely available

Deliverable D7.2 – Digital Tool

Deliverable Number	D7.2	**Lead Beneficiary**	12 - ACT
Deliverable Name	Digital Tool		
Type	OTHER	**Dissemination Level**	PU - Public
Due Date (month)	30	**Work Package No**	WP7

Description
The digital linkworking tool to support link worker as a open source software

Deliverable D8.1 – Study Initiation Package

Deliverable Number	D8.1	**Lead Beneficiary**	1 - Charite
Deliverable Name	Study Initiation Package		
Type	R — Document, report	**Dissemination Level**	PU - Public
Due Date (month)	14	**Work Package No**	WP8

Description
Study Initiation Package contains registration number of the trial, the study protocol and the regulatory and ethics approvals required for the enrolment of the first study participant

Deliverable D8.2 – GP Practice Recruitment Report

Deliverable Number	D8.2	**Lead Beneficiary**	1 - Charite
Deliverable Name	GP Practice Recruitment Report		
Type	R — Document, report	**Dissemination Level**	PU - Public
Due Date (month)	18	**Work Package No**	WP8

Description
Report after full recruitment of all GP practices as study sites. Including an overview of the number of recruited sites by RCT hub, any problems in recruitment and, if applicable, a detailed description of implemented and planned measures to compensate for any incurred delays.

Deliverable D9.1 – Midterm Recruitment Report

Deliverable Number	D9.1	**Lead Beneficiary**	1 - Charite
Deliverable Name	Midterm Recruitment Report		
Type	R — Document, report	**Dissemination Level**	PU - Public
Due Date (month)	23	**Work Package No**	WP9

Description
Report after 50% of recruited participants. Including an overview of the number of recruited participants by study sites, any problems in recruitment and, if applicable, a detailed description of implemented and planned measures to compensate for any incurred delays.

Deliverable D10.1 – Final Follow-Up Report

Deliverable Number	D10.1	**Lead Beneficiary**	1 - Charite
Deliverable Name	Final Follow-Up Report		
Type	R — Document, report	**Dissemination Level**	PU - Public
Due Date (month)	36	**Work Package No**	WP10

Description

Report after the last-patient-out of the trial. Including an overview of the number of recruited and interviewed participants by study sites and time points, any problems in follow-up and, if applicable, a detailed description of implemented measures to compensate for any incurred delays.

Deliverable D11.1 – Report on the status of posting trial results

Deliverable Number	D11.1	**Lead Beneficiary**	1 - Charite
Deliverable Name	Report on the status of posting trial results		
Type	R — Document, report	**Dissemination Level**	SEN - Sensitive
Due Date (month)	60	**Work Package No**	WP11

Description
Report on the scientific publications regarding the endpoints of the trial and their status and availability as preprint

Deliverable D11.2 – Anonymized Trial Data

Deliverable Number	D11.2	**Lead Beneficiary**	1 - Charite
Deliverable Name	Anonymized Trial Data		
Type	DATA — data sets, microdata, etc	**Dissemination Level**	SEN - Sensitive
Due Date (month)	60	**Work Package No**	WP11

Description
Anonymized trial data from baseline and Six-months and 12-months-follow up including extensive meta-data

Deliverable D12.1 – Trial Fidelity Report

Deliverable Number	D12.1	**Lead Beneficiary**	4 - UoP
Deliverable Name	Trial Fidelity Report		
Type	R — Document, report	**Dissemination Level**	PU - Public
Due Date (month)	48	**Work Package No**	WP12

Description
Report on the fidelity of the trial

Deliverable D13.1 – Theory of Change Model

Deliverable Number	D13.1	**Lead Beneficiary**	9 - BUW
Deliverable Name	Theory of Change Model		
Type	OTHER	**Dissemination Level**	PU - Public
Due Date (month)	18	**Work Package No**	WP13

Description
Explicated theory of change model of the intervention

Deliverable D13.2 – Health Economic Publication Update

Deliverable Number	D13.2	**Lead Beneficiary**	9 - BUW
Deliverable Name	Health Economic Publication Update		
Type	R — Document, report	**Dissemination Level**	SEN - Sensitive
Due Date (month)	54	**Work Package No**	WP13

Description
Report on the scientific publications regarding the health economic analysis of the trial and their status and availability as preprint

Deliverable D14.1 – Qualitative Recruitment Report

Deliverable Number	D14.1	**Lead Beneficiary**	10 - UOXF
Deliverable Name	Qualitative Recruitment Report		
Type	R — Document, report	**Dissemination Level**	PU - Public
Due Date (month)	23	**Work Package No**	WP14

Description
Report after all stakeholders for qualitative interviews have been recruited participants. Including an overview of the number of recruited participants by countries, any problems in recruitment and, if applicable, a detailed description of implemented measures to compensate for any incurred delays.

Deliverable D14.2 – Qualitative Study Publication Update

Deliverable Number	D14.2	**Lead Beneficiary**	10 - UOXF
Deliverable Name	Qualitative Study Publication Update		
Type	R — Document, report	**Dissemination Level**	SEN - Sensitive
Due Date (month)	36	**Work Package No**	WP14

Description
Report on the scientific publications regarding the comparative qualitative study and their status and availability as preprint

Deliverable D15.1 – Co-Interpretation Report

Deliverable Number	D15.1	**Lead Beneficiary**	7 - UNL
Deliverable Name	Co-Interpretation Report		
Type	R — Document, report	**Dissemination Level**	PU - Public

Due Date (month)	58	**Work Package No**	WP15

Description
Report on the conducted workshops with representatives of vulnerable groups and health care providers to co-interpret the results of SP-EU

Deliverable D15.2 – Co-Creation Publication Update

Deliverable Number	D15.2	**Lead Beneficiary**	8 - AAU
Deliverable Name	Co-Creation Publication Update		
Type	R — Document, report	**Dissemination Level**	SEN - Sensitive
Due Date (month)	58	**Work Package No**	WP15

Description
Report on the scientific publications regarding the co-creation and co-interpretation process and their status and availability as preprint

Deliverable D16.1 – Website incl. Knowledge Hub

Deliverable Number	D16.1	**Lead Beneficiary**	3 - concentris
Deliverable Name	Website incl. Knowledge Hub		
Type	DEC —Websites, patent filings, videos, etc	**Dissemination Level**	PU - Public
Due Date (month)	5	**Work Package No**	WP16

Description
A website to support communication and dissemination of SP-EU

Deliverable D16.2 – Communication and Dissemination Plan - V1

Deliverable Number	D16.2	**Lead Beneficiary**	17 - NASP
Deliverable Name	Communication and Dissemination Plan - V1		
Type	R — Document, report	**Dissemination Level**	PU - Public
Due Date (month)	6	**Work Package No**	WP16

Description
The detailed preliminary communication and dissemination plan of SP-EU including target groups and communication and dissemination channels and measures

Deliverable D17.1 – Report on the Use of Communication Channels

Deliverable Number	D17.1	**Lead Beneficiary**	17 - NASP

Deliverable Name	Report on the Use of Communication Channels		
Type	R — Document, report	**Dissemination Level**	PU - Public
Due Date (month)	27	**Work Package No**	WP17

Description
Report on the use of communication channels such as the website and social media during the first 24 months of the project

Deliverable D17.2 – Communication and Dissemination Plan - V2

Deliverable Number	D17.2	**Lead Beneficiary**	17 - NASP
Deliverable Name	Communication and Dissemination Plan - V2		
Type	R — Document, report	**Dissemination Level**	PU - Public
Due Date (month)	30	**Work Package No**	WP17

Description
Communication and Dissemination plan updated for the second reporting period.

Deliverable D18.1 – Report on the Established Dissemination Networks

Deliverable Number	D18.1	**Lead Beneficiary**	15 - EuroHealthNet
Deliverable Name	Report on the Established Dissemination Networks		
Type	R — Document, report	**Dissemination Level**	PU - Public
Due Date (month)	39	**Work Package No**	WP18

Description
Report on which networks for dissemination have been established in the first 36 months, e.g., with other projects funded under the same call and related European projects

Deliverable D18.2 – Explanatory Film on SP-EU for the Public

Deliverable Number	D18.2	**Lead Beneficiary**	17 - NASP
Deliverable Name	Explanatory Film on SP-EU for the Public		
Type	DEC —Websites, patent filings, videos, etc	**Dissemination Level**	PU - Public
Due Date (month)	48	**Work Package No**	WP18

Description
A film produced on the results of SP-EU in an easy understandable language

Deliverable D18.3 – Communication and Dissemination Plan - V3

Deliverable Number	D18.3	**Lead Beneficiary**	15 - EuroHealthNet
Deliverable Name	Communication and Dissemination Plan - V3		
Type	R — Document, report	**Dissemination Level**	PU - Public
Due Date (month)	48	**Work Package No**	WP18

Description
Communication and Dissemination plan updated for the third reporting period.

Deliverable D19.1 – Report on Policy Roundtables

Deliverable Number	D19.1	**Lead Beneficiary**	15 - EuroHealthNet
Deliverable Name	Report on Policy Roundtables		
Type	R — Document, report	**Dissemination Level**	PU - Public
Due Date (month)	60	**Work Package No**	WP19

Description
Report on the national and European level roundtables, their participants and results

Deliverable D19.2 – Final Exploitation Plan

Deliverable Number	D19.2	**Lead Beneficiary**	15 - EuroHealthNet
Deliverable Name	Final Exploitation Plan		
Type	R — Document, report	**Dissemination Level**	PU - Public
Due Date (month)	60	**Work Package No**	WP19

Description
Final exploitation plan to facilitate the exploitation of SP-EU towards the highest possible impact after the end of the project

Deliverable D19.3 – Communication and Dissemination Plan - V4

Deliverable Number	D19.3	**Lead Beneficiary**	15 - EuroHealthNet
Deliverable Name	Communication and Dissemination Plan - V4		
Type	R — Document, report	**Dissemination Level**	PU - Public
Due Date (month)	60	**Work Package No**	WP19

Description
Communication and Dissemination plan updated for the fourth reporting period.

LIST OF MILESTONES

Milestones

Grant Preparation (Milestones screen) — Enter the info.

Milestone No	Milestone Name	Work Package No	Lead Beneficiary	Means of Verification	Due Date (month)
1	Kick-Off Meeting	WP1	1 - Charite	Minutes of the 1st GA	3
2	Co-created adapted social prescribing intervention	WP5	8 - AAU	Report on the adapted intervention	12
3	Approval of trial by lead ethics committee	WP8	1 - Charite	Approval letter	14
4	First Patient In	WP9	1 - Charite	Documentation in eCRF	17
5	Last Patient Out	WP10	1 - Charite	Documentation in eCRF	42
6	Primary endpoint analysed	WP11	1 - Charite	Analysis report	45
7	Co-Interpretation workshops conducted	WP15	7 - UNL	Minutes of workshops	54
8	Final symposium conducted	WP19, WP4	1 - Charite	Documentation of symposium	60

LIST OF CRITICAL RISKS

Critical risks & risk management strategy

Grant Preparation (Critical Risks screen) — Enter the info.

Risk number	Description	Work Package No(s)	Proposed Mitigation Measures
1	Difficulties in interdisciplinary collaboration involving a large number of partners (Likelihood: Medium/ Severity: Medium)	WP2, WP3, WP1, WP4	We will work right from the start on team building and a common understanding. We will dedicate specific time slots for both aspects in each general assembly. We will develop a

Critical risks & risk management strategy

Grant Preparation (Critical Risks screen) — Enter the info.

Risk number	Description	Work Package No(s)	Proposed Mitigation Measures
			glossary of terms to enable a shared understanding. In case of misunderstandings, we will set up a timely online call and if necessary also additional in person meetings
2	Difficulties to find representatives of people in vulnerable situations for co-creation and co-interpretation (Likelihood: Medium/ Severity: High)	WP15, WP5	In the consortium, there are partners representing each of the three groups. These partners will help the consortium to find representatives for the co-creation profess. Additionally, we have already involved representatives in writing the proposal which we will invite to take part in the co-creation process as well. Each of the three lead partners in this WP as well as other partners in the consortium have extensive experience in working with these groups of persons in vulnerable situations. Thus, we can rely on these existing networks of representatives for the co-creation process.
3	The digital linkworking tool is not finished on time to be used in the trial (Likelihood: Low/ Severity: High)	WP6, WP7	The agile project management of ACT provides by incremental procedures early in the process applicable software. In the worst-case scenario, we can conduct the trial also without the software.
4	One of the RCT hubs leaves the consortium due to unforeseen circumstances (Likelihood: Low/ Severity: High)	WP9, WP8, WP11, WP10	We do have a back-up hub at the Department of Preventive Medicine, Charles University, Faculty of Medicine in Hradec Kralove, Czech Republic, which could replace an RCT hub in case it leaves the consortium.
5	Delays in achieving ethics approvals in some countries involved (Likelihood: Medium/ Severity: Medium)	WP2, WP3, WP8, WP1, WP4, WP14	To mitigate this risk, we will start each study part as soon as a first “pioneer” partner has received ethical approval. Additionally, we will have a position at the coordinator who will help each partner to receive ethics approval. The partners will support each other by sharing their documents and experiences.
6	Low recruitment of trial participants in one of the hubs (Likelihood: Medium/ Severity: Medium)	WP9, WP10	The assumptions for recruitment are conservative. Underrecruiting by one partner could be mitigated by higher recruitment of the other partners and we would reallocate the funding accordingly. Additionally, we have planned the critical path, so we extend the recruitment period if necessary.
7	Low recruitment of trial participants in one of the study baskets (i.e., one of the three groups) (Likelihood: Medium/ Severity: Medium)	WP9, WP10	In case we monitor a too low recruitment in one study basket, we will a) involve the local advisory boards to assess potential reasons and solutions and b) try to recruit purposively GP practice which treat a higher share of patients from this group.
8	Not enough trial stakeholders agree to being	WP12	In case of under recruitment of qualitative interview partners in the process evaluation,

Critical risks & risk management strategy

Grant Preparation (Critical Risks screen) — Enter the info.

Risk number	Description	Work Package No(s)	Proposed Mitigation Measures
	interviewed for the process evaluation (Likelihood: Low/ Severity: Medium)		we will intensify the recruitment by personal visits of hubs and sites. Formal qualitative interviews can be enriched by participant observation on-site.
9	The economic analysis reveals that a framework for sustainable financing of SP is not feasible (Likelihood: Low/ Severity: Medium)	WP13	In case the economic analysis reveals that a sustainable financing of SP on the long run is not feasible, we will take this result up and communicate it in policy briefings and policy roundtables proactively.
10	One of the partners involved in the qualitative part leaves the consortium due to unforeseen circumstances (Likelihood: Low/ Severity: Medium)	WP14	IDIAPJGol serves as qualitative back-up hub and could replace a qualitative partner who left the consortium.
11	Target number of interviews is not reached in the qualitative study part (Likelihood: Low/ Severity: Medium)	WP14	If we detect a too low recruitment, we can involve partners from other WPs such as WONCA Europe, NASP and EuroHealthNet to help finding additional interviewees.
12	The communication messages on social prescribing are perceived as threats by stakeholders involved, e.g., because they fear financial burden for health insurances (Likelihood: Low/ Severity: Medium)	WP18, WP16, WP19, WP17	We will prepare communication messages involving stakeholders early in the process to frame them in the best possible way. In case of misunderstandings, out communication team will react promptly with on the point communication also including the PR departments of the partners involved in SP-EU.

PROJECT REVIEWS

Project Reviews *Grant Preparation (Reviews screen) — Enter the info.*			
Review No	**Timing** (month)	**Location**	**Comments**
RV1	14	TBD	Subject to approval by PO. Linked with 1st periodic reporting.
RV2	32	TBD	Subject to approval by PO. Linked with 2nd periodic reporting.
RV3	50	TBD	Subject to approval by PO. Linked with 3rd periodic reporting.
RV4	62	TBD	Subject to approval by PO. Linked with 4th periodic reporting.